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Generational gap regarding patients' driving habits

To the Editor: The article, "Sunscreen use while driving,"[1] serves as a valuable reminder of the importance of proper patient education regarding sunscreen use. The authors found that only 26% of the patients surveyed used sunscreen at least half the time while in the car. They drove the importance home by noting that 62% of the nonmelanoma skin cancers in their patients developed on the left side, compared with 28% on the right side ($P < .03$). This is a significant difference to note, especially considering that clear-glass and dark-tinted windows in cars only transmit 62% and 11.4% of ultraviolet (UV) A, respectively.

However, it is important to discuss a likely generational gap that exists between the patients they surveyed and current young drivers. The average patient age in the survey was 67.5 years, which means the majority of patients started their main driving years in the early 1960s. This is important since only in 1968, with the production of the American Motors Corporation's Ambassador, was air conditioning made standard in automobiles.[2] In 1969, only 54% of domestic automobiles were equipped with air conditioning.[3] In addition, 42.4% of adults were smokers in 1965, compared to 20.6% in 2008.[4] Considering both of these factors, it is likely that at least some of the patients surveyed drove with the windows down far more often than current drivers do, resulting in more exposure to UV radiation, particularly UVB, which is blocked by window glass. We thought this generational gap also deserved comment, as it may further have contributed to the effects of driving on UV light exposure and the left-sided dominance of actinic lesions in this US driving population.

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High survival rate of harlequin ichthyosis in Japan

To the Editor: Harlequin ichthyosis (HI) (OMIM 242500) is one of the most severe genetic skin disorders, and its clinical features at birth include severe ectropion, eclabium, flattening of the ears, and large, thick platelike scales over the entire body. Neonatal death is common.

We read with great interest the recent commentary by Milstone and Choate[1] on the importance of intensive neonatal care in improving HI outcomes. In their commentary, the authors conducted a critical examination of the intrinsic and extrinsic factors contributing to the prognosis of HI and they asserted that there are many effective measures for improving the outcomes of babies with HI.[1] For example, respiratory failure frequently contributes to neonatal death in HI[2]; thus, they noted that perhaps intubation should have been done in more cases.[1] A multicenter retrospective questionnaire-based survey in England, Sweden, the United States, Iran, Turkey, and New Zealand reported an overall survival rate of 56% (25 of 45 patients).[2] Eighty-three percent of HI neonates treated with systemic retinoids survived, whereas the long-term survival was only 24% for those who were not given oral retinoids.[2]

Here we report the outcomes for HI in the Japan population. For a clinical survey of HI in Japan, we distributed questionnaires to 904 dermatologic or pediatrics institutes or hospitals throughout Japan in 2010 and received responses from 564 institutes or hospitals (62.4%). Clinical data between 2005 and 2010 were obtained for 16 HI patients. In total, there were 13 survivors (81.3%) and 3 deceased (18.7%). The patient's sex was reported for 14 patients: 8 male and 6 female. Systemic retinoids were administered to 12 patients, 11 of whom survived (91.7%), whereas only 2 of the 4 patients (50%) who did not receive oral retinoids survived. Ten of 16 patients (62.5%) received intensive care in a neonatal intensive care unit (NICU).

Systemic retinoids and administration in the NICU were considered to contribute to relatively good outcomes for HI patients in the Japanese population.

The effects of oral retinoids in HI remains unproven,[3] and as suggested in the commentary by Milstone and Choate,[1] the early introduction of overall intensive therapy might have contributed to the better outcomes of the HI babies who were given oral retinoids.

In conclusion, we consider that, due to intensive neonatal care and, probably, to the early introduction of oral retinoids, HI outcomes have improved also in the Japanese population. In long-term HI survivors, epidermal keratinocytes might regain normal differentiation, and this restoration of differentiation is likely to be associated with the improvement of the skin symptoms in HI survivors.[4] From this fact and the results of our clinical survey, we agree with the commentary by Milstone and Choate[1] on the point that recognition of spontaneous improvement of the HI phenotype lowers the psychological hurdle and provides justification for intensive neonatal care that can improve the outcomes of HI patients.

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Research letters

Retronychia in children, adolescents, and young adults: A case series

To the Editor: The term *retronychia* describes the embedding of the proximal nail plate (NP) into the proximal nail fold (PNF) and has been reported mainly in adults.[1] We report here a case series of retronychia in children younger than 12 years of age, adolescents aged 13 to 18 years, and young adults aged 19 to 24 years.

All cases were discussed by dermatologists from 4 different centers. All patients gave informed consent for photographs and were followed